SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Gemcitabine + carboplatin AUC2

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Locally recurrent or metastatic triple-negative breast cancer · NCCP 00430 · version 6 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control advanced cancer and symptoms, with regular review of benefit and side effects.

Schedule
Carboplatin AUC2 is given first, followed by gemcitabine 1,000 mg/m², on BOTH Days 1 and 8 of each 21-day cycle. Treatment continues until progression or unacceptable toxicity.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Allergic reaction, sometimes after previous exposure
- ☐ Kidney or electrolyte changes

Gemcitabine
ANTIMETABOLITE CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Flu-like symptoms, fatigue, rash and swelling
- ☐ Uncommon serious lung or liver injury
- ☐ Rare haemolytic uraemic syndrome and kidney injury

Discuss the treatment setting, alternatives, exact schedule, common and serious drug-specific risks, fertility and pregnancy precautions, and the monitoring and urgent-contact plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00430 v6.

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